

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase 3 Study of Pembrolizumab (MK-3475) Versus Chemotherapy in Chinese Participants With Stage IV Microsatellite Instability-High (MSI-H) or Mismatch Repair Deficient (dMMR) Colorectal Cancer (MK-3475-C66)
Short Title:	Study of Pembrolizumab (MK-3475) Versus Chemotherapy in Chinese Participants With Stage IV Colorectal Cancer (MK-3475-C66)
Protocol Number:	685802075566128369
NCT Number:	NCT05239741
Sponsor Name:	Merck
Investigational Product:	fluorouracil
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Progression-Free Survival (PFS) per Response Evaluation Criteria in Solid Tumors Version 1.1 (RECIST 1.1) as Assessed by Blinded Independent Central Review (BICR) For All Participants: PFS is defined as the time from randomization to the first documented disease progression (PD) per RECIST 1.1 or death due to any cause, whichever occurs first. Per RECIST 1.1, PD is defined as ≥20% increase in the sum of diameters of target lesions. In addition to the relative increase of 20%, the sum must also demonstrate an absolute increase of ≥5 mm. Note: The appearance of one or more lesions and the unequivocal progression of non-target lesions is also considered PD. The PFS per RECIST 1.1 as assessed by BICR will be reported for all participants. 2. Progression-Free Survival (PFS) per Response Evaluation Criteria in Solid Tumors Version 1.1 (RECIST 1.1) as Assessed by Blinded Independent Central Review (BICR) For RAS Wild-type Participants: PFS is defined as the time from randomization to the first documented disease progression (PD) per RECIST 1.1 or death due to any cause, whichever occurs first. Per RECIST 1.1, PD is defined as ≥20% increase in the sum of diameters of target lesions. In addition to the relative increase of 20%, the sum must also demonstrate an absolute increase of ≥5 mm. Note: The appearance of one or more lesions and the unequivocal progression of non-target lesions is also considered PD. The PFS per RECIST 1.1 as assessed by BICR will be reported for RAS wild-type participants .
Secondary Obj.:	1. Overall Survival (OS) For All Participants 2. Overall Survival (OS) For RAS Wild-type Participants 3. Objective Response Rate (ORR) per Response Evaluation Criteria in Solid Tumors Version 1.1 (RECIST 1.1) as Assessed by Blinded Independent Central Review (BICR) For All Participants

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2.2 Background & Rationale

In this study, Chinese participants with MSI-H or dMMR advanced colorectal cancer will be assigned to receive either pembrolizumab or the Investigator's choice of 1 of 6 standard of care (SOC) chemotherapy regimens for treatment. There is no hypothesis testing for this study.

3. Study Design & Methodology

Design Framework: Type: INTERVENTIONAL, Phase: PHASE3, Status: RECRUITING

Target N: 100

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

Inclusion Criteria include, but are not limited to:

- * Has a histologically confirmed diagnosis of colorectal adenocarcinoma that is at stage IV (as defined by American Joint Committee on Cancer eighth edition) \[National Comprehensive Cancer Network 2018\]
- * Has centrally confirmed Microsatellite Instability-High/Mismatch Repair Deficient (MSI-H/dMMR) status
- * Has centrally confirmed RAS and BRAF mutation status
- * A woman of child-bearing potential (WOCBP) must have a negative highly sensitive pregnancy test (urine or serum as required by local regulations) within 24 hours for urine or within 72 hours for serum before the first dose of study intervention.
- * Has measurable disease per Response Evaluation Criteria In Solid Tumors (RECIST) 1.1 as assessed by the local site investigator/radiology
- * Must provide an archival tumor tissue sample or newly obtained core or excisional biopsy of a tumor lesion not previously irradiated.
- * Has an Eastern Cooperative Oncology Group (ECOG) performance status of 0 to 1 within 3 days before randomization
- * Has a life expectancy of at least 3 months
- * Has received Hepatitis B Virus (HBV) antiviral therapy for at least 4 weeks, and have undetectable hepatitis B virus (HBV) viral load prior to randomization if hepatitis B surface antigen (HBsAg) positive
- * Has an undetectable Hepatitis C Virus (HCV) viral load if HCV infected
- * Has well controlled Human Immunodeficiency Virus (HIV) on antiretroviral therapy (ART) if HIV infected

4.2 Exclusion Criteria

Exclusion Criteria include, but are not limited to:

- * Has received prior systemic therapy for stage IV colorectal cancer (CRC). Participants may have received prior adjuvant/neoadjuvant chemotherapy for CRC as long as it was completed at least 6 months prior to randomization
- * Has undergone major operation within 4 weeks of randomization or has not adequately recovered from major surgery or has ongoing surgical complications
- * Has received prior therapy with an anti-programmed cell death 1 protein (anti-PD-1), anti-programmed cell death ligand 1 (anti-PD-L1), or anti-programmed cell death ligand 2 (anti PD-L2) agent or with an agent directed to another

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stimulatory or coinhibitory T-cell receptor (eg, cytotoxic T-lymphocyte-associated protein 4 \[CTLA-4\], tumor necrosis factor receptor superfamily, member 4 \[OX 40\], tumor necrosis factor receptor superfamily member 9 \[CD137\])

- * Has received prior radiotherapy within 2 weeks of start of study intervention or has radiation-related toxicities, requiring corticosteroids
- * Has received a live or live-attenuated vaccine within 30 days before the first dose of study intervention
- * Has received an investigational agent or has used an investigational device within 4 weeks prior to study intervention administration.
- * Has a diagnosis of immunodeficiency or is receiving chronic systemic steroid therapy (in dosing exceeding 10 mg daily of prednisone equivalent) or any other form of immunosuppressive therapy within 7 days prior to the first dose of study medication
- * Has a known additional malignancy that is progressing or has required active treatment within the past 5 years, with the exception of basal cell carcinoma of the skin, squamous cell carcinoma of the skin, or carcinoma in situ (eg, breast carcinoma, cervical cancer in situ) that have undergone potentially curative therapy are not excluded
- * Has known active central nervous system (CNS) metastases and/or carcinomatous meningitis
- * Has severe hypersensitivity (?Grade 3) to pembrolizumab and/or any of its excipients
- * Has an active autoimmune disease that has required systemic treatment in past 2 years
- * Has a history of (noninfectious) pneumonitis/interstitial lung disease that required steroids or has current pneumonitis/interstitial lung disease
- * Has an active infection requiring systemic therapy (eg, tuberculosis, known viral or bacterial infection)
- * Has HIV-infection with a history of Kaposi's sarcoma or Multicentric Castleman's Disease
- * Has had an allogenic tissue/solid organ transplant

11. Study Identifiers & Governance

Primary ID: 685802075566128369

Registry Number: NCT05239741

Sponsor: Merck

Study Title: Study of Pembrolizumab (MK-3475) Versus Chemotherapy in Chinese Participants With Stage IV Colorectal Cancer (MK-3475-C66)

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____